Medical Records - CONFIDENTIAL

FROM: TX - Texas Colon and Rectal Surgeons

Cindy G
4207 JAMES CASEY ST STE 201 BLDG A, AUSTIN, TX 78745-1193
Phone: (512) 447-8911
Fax: (512) 447-8761

TO:

Name: DESCROIX, RICHARD

DOB: 03/21/1956

Date Range: 01/01/2026 to 03/03/2026

This document contains the following records of the patient:
- **Clinical Documents**

Note: RICHARD DESCROIX

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DESCROIX, Richard (id #872031, dob: 03/21/1956)

Clinical Documents

DESCROIX, Richard (id #872031, dob: 03/21/1956)

click here to view original file

PAM HEALTH AT HOME

Fax

To: SOLIS, RICARDO
From: Pam Health at Home - Texas
Fax: 18554271214
Pages: 8
Phone:
Date: 02-27-2026 6:10 PM
Re: Order(s) for Signature
Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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PAM Health at Home
825 Main St Ste 110, Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

ROC Order

Order No: R-24904 **Order Date:** 01/27/2026 11:48 AM

Patient

Descroix, Richard (MR#: 7702)
456 Betony Loop
Buda, TX, 78610
(760) 750-2521
DOB: 03/21/1956

Physician

RICARDO SOLIS MD
4007 James Casey Drive Ste D150
AUSTIN, TX, 78745
(512) 447-8911 Fax: (855) 427-1214
NPI: 1891802732

Payer Source: PALMETTO GBA
Primary Diagnosis: Z48.3 - Aftercare following surgery for neoplasm
Certification Period: 12/26/2025-02/23/2026
Requested Staff: Byrnes, Sarah - RN

Order Description

SCHEDULE:
SN-SN Frequency: 1W4 Starting Week of 01/25/2026,
SN PRN Visits/Hours: 3 (Ostomy complications)
PT-PT Frequency: 1W1 EVAL AND TREAT Starting Week of 01/25/2026,

Medications:
Ferrous sulfate 325 mg (65 mg iron) tablet(Route=oral, Frequency=daily, Dosage=325mg, Purpose=supplement, amount Of= 1.00)

Vitamin D2 20 mcg-vitamin K1 120 mcg/4 drops oral(Route=oral, Frequency=daily, Dosage=120mcg, Purpose=supplement)

Staff/Clinician Signature: Byrnes, Sarah - RN **Date:** 01/27/2026 **Time:** 11:48 AM
Electronically signed

ROC Order

Patient: Descroix, Richard (MR#: 7702) Order No:R-24904

Patient DOB: 03/21/1956 Order Date: 01/27/2026 11:48 AM

Probiotic 10 billion cell capsule(Route=oral, Frequency=daily, Dosage=10 billion , Purpose=supplement)

Advil 200 mg tablet(Route=oral, Frequency=2-3 times a day as needed, Dosage=1-2 , Purpose=pain, PRN/As Needed=Yes)

Acetaminophen 500 mg capsule(Route=oral, Frequency=every 4-6 hours as needed, Dosage=1-2, Purpose=pain, PRN/As Needed=Yes)

Amoxicillin 875 mg-potassium clavulanate 125 mg tablet(Route=oral, Frequency=every 12 hours x 6 days, Dosage=1)

Tramadol 50 mg tablet(Route=oral, Frequency=every 6 hrs as needed, Dosage=1, Purpose=pain, PRN/As Needed=Yes)

Lidocaine 4 % topical patch(Route=topical, Frequency=daily as needed, Dosage=1, Purpose=pain, PRN/As Needed=Yes)

Interventions:

BEST PRACTICE FALL PREVENTION
Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION
Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN
Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain

GENITOURINARY

Staff/Clinician Signature: Byrnes, Sarah - RN Date: 01/27/2026 Time: 11:46 AM
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ROC Order

Patient: Descroix, Richard (MR#: 7702) Order No:R-24304

Patient DOB :03/21/1956 Order Date: 01/27/2026 11:46 AM

Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.

OSTOMIES
Clinician to assess/Perform/instruct patient/caregiver on colostomy care using aseptic technique as follows:
Stoma: Remove appliance with adhesive remover. Cleanse with water. Apply barrier film. Apply stoma ring. Apply pouch - Hollister 2pc flat appliance. Empty pouch when 1/3 to 1/2 full. Change pouch if experiencing any burning or itching. Stoma barrier to be cut 1/8" larger than stoma.
SN to instruct patient/caregiver on ostomy management as follows: Showering and bathing will not hurt your ostomy or the appliance. Some people prefer to shower without their appliance on the days they change their pouching system. You can decide what you prefer. If you use soap make sure it does not contain any oils or lotions as this can interfere with how secure y your faceplate adheres to your skin. Be sure to rinse the skin well. The residue from the soap could prevent the faceplate from sticking as well. Dry your skin gently but well before applying the new faceplate.
SN to instruct patient/caregiver on ostomy management as follows: Change your pouch every 4-7 days.
SN to instruct patient/caregiver on ostomy management as follows: Eat 5-6 small meals a day. Ensure your diet is well balanced. Avoid skipping meals since this may cause diarrhea and gas. Introduce one new food at a time to see the effect on your body. A food journal may be helpful. Chew all foods well. Cook foods well until they are tender. Be mindful of foods that may increase gas, odor, stoma obstruction, diarrhea, or constipation. When the colon is removed to create an ileostomy there is a greater risk for dehydration and electrolyte imbalance. This is because the colon usually absorbs the majority of the fluids and electrolytes the body needs to function.
Diarrhea, vomiting, or perspiration can increase the risk of dehydration and electrolyte imbalances. To prevent fluid and electrolyte imbalances drink 8-12 cups of fluid per day. Avoid using straws(will increase gas). Drink caffeinated beverages moderately as they can lead to dehydration.
SN to instruct patient/caregiver on ostomy management as follows: Empty your pouch when it is 1/3 to 1/2 full. A full pouch can get heavy and cause the appliance to lose a good seal and leak.
SN to instruct patient/caregiver on ostomy management as follows: Make sure you have enough appliances and ostomy supplies on hand to last you for three to four weeks. Carry an extra appliance with you at all times. If you are admitted to a hospital, take your ostomy supplies with you. The hospital may not stock the brand or size appliance you use.
SN to instruct patient/caregiver on ostomy management as follows: Whenever possible, stand when applying your ostomy appliance. This will help keep the skin around your ostomy smooth and flat, and will improve the quality of the seal you get between your faceplate and the skin.

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Patient: Descroix, Richard (**MRN**: 7702) **Order No:**R-24904

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SURGICAL WOUND
Clinician to assess surgical wound, instruct/perform wound care as follows: Leave Lt upper chest surgical site open to air and report any redness or dehiscence to SN/MD

HOSPITALIZATION RISK FACTORS
Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: perforated bowel s/p resection with new colostomy, septic shock, colorectal cancer, anemia.
Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (w/diet), etc
Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

Goals:

BEST PRACTICE FALL PREVENTION
LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION
LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode

BEST PRACTICE MONITOR AND MITIGATE PAIN
LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

GENITOURINARY
LTG-Patient will tolerate collection of urine without pain and discomfort for S/Sx UTI by end of episode.

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OSTOMIES
STG-Patient/caregiver will demonstrate ability to perform ostomy care independently as ordered by 4 weeks.
LTG-Patient's colostomy will remain free of infection/breakdown by end of episode.

SURGICAL WOUND
LTG-Patient/Caregiver will be able to identify 3 s/s of infection of surgical wound by end of episode.

HOSPITALIZATION RISK FACTORS
Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

DME:
Wheel Chair,Walker,Other (specify): ostomy supplies, ALCOHOL WIPES, NS GLOVES, PROBE COVERS, PPE MASK

Safety measures:
911 Emergency / Protocol,Assistance with IADLs,Emergency Plan / Measures Emergency Plan Developed Equipment Safety,Fall / Safety Precautions,Hazard-Free Environment,Infection Control Standard Measures and Precautions,Keep Pathways Clear and Safe,Medication Safety / Storage,Proper Hazardous Waste Disposal,Safety in ADLs,Skin Care,Slow Position Change,Support During Transfer and Ambulation,Use of Assistive Devices,Walker / Cane

Nutritional Requirements:
Regular,Protein Hi Diet
Prognosis:
Good
Mental Status:
Oriented
Functional Limitations:
Bowel/Bladder(incontinence),Hearing,Endurance,Ambulation

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Patient: Descroix, Richard (MR#: 7702) Order No:R-24904

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Activities Permitted:
Exercises Prescribed,Cane,Walker,Other: SBA
Diagnosis Codes:
Z48.3 - Aftercare following surgery for neoplasm
C20 - Malignant neoplasm of rectum
K63.1 - Perforation of intestine (nontraumatic)
Z43.3 - Encounter for attention to colostomy
G72.89 - Other specified myopathies
Z45.2 - Encounter for adjustment and management of vascular access device
D64.9 - Anemia, unspecified
E87.0 - Hyperosmolality and hypernatremia
E87.6 - Hypokalemia
N40.0 - Benign prostatic hyperplasia without lower urinary tract symptoms
Z79.891 - Long term (current) use of opiate analgesic

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation pm sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days.
I attest to the following:
1. RICHARD DESCROIX is under my care.
2. I have received the following patient records from Be Healthy at Home:
a. Patient Summary Form

Staff/Clinician Signature: Byrnes, Sarah - RN Date: 01/27/2026 Time: 11:48 AM
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ROC Order

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b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.
4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

Comments

Staff/Clinician Signature: Electronically signed Byrnes, Sarah - RN **Date:** 01/27/2026 **Time:** 11:48 AM

Physician Signature: **Date:** 3/2/26